

A PROPOSAL FOR

Operational and leadership support for Medbury Healthcare

Getting the Chief Executive out of operations and the sector leaders into their seats.

PREPARED FOR

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PREPARED BY

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DATE

April 2026

STATUS

Private and confidential. NDA available on request.



A NOTE FROM DEBO

Picking up where we left off.

Itunu, when we spoke a couple of weeks ago, the picture you painted was familiar: a capable Chief Executive being pulled into operations because the bench underneath isn't holding. One strong sector lead carrying more than her share. The other sectors either under-performing or quietly draining your time.

I promised to help. This document is what that help looks like: short on paper, weighted towards action. It's also a reminder that you don't have to fix this alone.

"At Consult for Africa we implement, we don't just advise. The binding constraint in most mid-sized healthcare groups isn't strategy or capital. It's the quality of the operating layer one level below the CEO. Fix that layer and the CEO gets their job back."

What's inside

- 1. How we see the problem.** A short reframe of the challenge you described.
- 2. Two tracks of support.** Leadership bench and operational diagnostics, designed to run in parallel.
- 3. Our execution model.** How Consult for Africa moves from diagnosis to sustained performance.
- 4. About us and next steps.** Who's behind this, and how to start.



01 / THE PROBLEM, REFRAMED

Three things are happening at once.

A thin leadership layer.

Only one sector lead is currently performing at the level you need. The others are either in post but under-delivering, or structurally absent. Every vacancy or weak seat routes escalation back to the CEO.

A CEO absorbing operational load.

When sector leads don't hold, the Chief Executive becomes the default Chief Operating Officer. That's expensive in three ways: your time, the decisions that don't get made because you're in the weeds, and the signal it sends to the rest of the organisation about who really owns outcomes.

No current read on where the real leverage is.

Without a structured diagnostic at each sector, it's genuinely hard to tell whether a sector is failing because of its leader, its systems, its market, or some combination. Hiring without that answer is expensive. You either over-pay for a saviour who inherits a broken system, or under-hire into a role that was always going to need senior weight.

The implication. Talent and diagnostics have to move together. Hire without understanding the operational reality, and you'll cycle through leaders. Diagnose without a talent pipeline ready, and insight won't convert into change.



02 / HOW WE CAN HELP

Two tracks, designed to run in parallel.

Each track stands on its own, but they are far stronger when sequenced together. The diagnostic sharpens the brief. The brief sharpens the hire. The hire sticks.

TRACK ONE

Leadership and embedded operators

Access to the Consult for Africa network of senior healthcare operators and sector leads: people who have actually run services at scale in this market.

What you get

- Role briefs co-developed with you: scope, scorecard, non-negotiables.
- Pre-screened shortlists per role, with warm introductions.
- Reference conversations handled discreetly through our network.
- Optional embedded-operator engagements to hold a seat until a permanent hire lands.

TRACK TWO

Performance diagnostic and turnaround

A structured review of one or two of your under-performing business units, followed by a prioritised intervention roadmap we can help you deliver.

What you get

- Independent read on what is actually broken versus what looks broken.
- Written findings and a prioritised intervention roadmap.
- A clear view on whether the sector needs a new leader, new systems, or both.
- Optional embedded execution to deliver the roadmap, not just hand it over.

A note on commercials. Track 1 is offered on a warm-introduction basis between us, no placement fees. Track 2 is a formal Consult for Africa engagement, scoped and priced against the specific units in view. We scope it together before anything starts.



03 / OUR EXECUTION MODEL

Five stages. One operating discipline.

Consult for Africa engagements follow a deliberate five-stage model. It is the same discipline we bring whether the engagement is a three-week diagnostic or a twelve-month embedded turnaround. The point is to leave the institution stronger, not dependent.



Diagnose	Independent, on-site read of the business unit: governance, clinical operations, revenue cycle, people, digital. Triangulated from interviews, observation, and data.
Design	Prioritised intervention roadmap co-owned with your leadership. Clear sequencing, accountability, and measurable outcomes.
Deploy	We move from paper to execution. Embedded operators, interim leadership, or programme management as the roadmap requires.
Deliver	Performance cadence installed: KPIs, dashboards, weekly operating rhythm. Outcomes measured against the baseline agreed at scoping.
Transfer	Capabilities handed to permanent leadership. We leave behind systems and habits that outlast the engagement.

For Medbury, a single-unit engagement typically sits inside a four-week Diagnose-and-Design window, with Deploy, Deliver, and Transfer scoped separately once findings are on the table.



04 / ABOUT AND NEXT STEPS

Who's behind this, and how to begin.

About Consult for Africa

Consult for Africa is an Africa-focused healthcare management and transformation firm supporting operators, investors, and institutions to strengthen performance, governance, and execution. The network brings together 60+ years of combined senior leadership: former hospital CEOs, clinicians who have led accreditation and quality programmes at scale, revenue-cycle and HMO specialists, and digital operators who have deployed HIS and EMR platforms in resource-constrained environments.

Our model is execution-led. We implement, we don't just advise.

About the lead

Debo is a senior healthcare executive with a track record spanning hospital commissioning, revenue growth, and digital transformation. He splits his time between Lagos and Abuja and brings a perspective shaped by running operations, not just advising on them. His engagement model on this work is personal: he leads the diagnostic and stays close throughout the intervention.

NEXT STEPS: three small moves to get started.

- 1. A 30-minute call.** To align on priorities, sequence the two tracks, and agree which business unit goes first.
- 2. Role briefs.** Send what you have, even rough drafts, for the sector-lead roles you're most concerned about. We'll start sourcing.
- 3. A scoping note.** We'll come back with a short scoping note for the Diagnose-and-Design phase: unit, access, timeline, fee.

Contact

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Executive response within 48 hours

This document is shared in confidence with Dr Itunu Akinware and Medbury Healthcare leadership for the purpose of discussing a potential engagement. It is not a binding offer; scope and commercials will be agreed in writing before any work begins. NDA available on request.